

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

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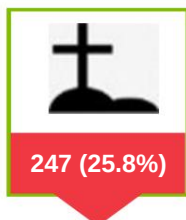
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°36/MVB_19/06/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (34)	• Ituri (22/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia and Fataki. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. • South Kivu (1/34): Miti-Murhesa
Reporting date	June 19, 2026
Publication date	June 20, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



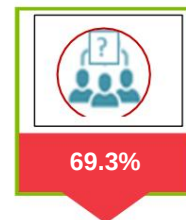
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- **Twenty-three (23) new confirmed cases, including 2 deaths**, were reported during the day on June 19, 2026, in the provinces of **Ituri (21, including 2 deaths) and North Kivu (2)**. These confirmed cases are distributed in the health zones of **Mongbwalu (11), Bunia (6), Nyankunde (2) and Rwampara (2)**, in the province of Ituri, as well as in the **Katwa (2)** health zone in the province of North Kivu.
- **Twelve (12) patients** with Ebola virus disease have been declared cured after receiving negative control tests.
- A working session was held between the national-level ministerial delegation, the provincial governor and the SGI MVE to assess the response activities.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a population

total estimated at nearly 15 million inhabitants, with massive internal displacements and cross-border flows to neighboring countries.

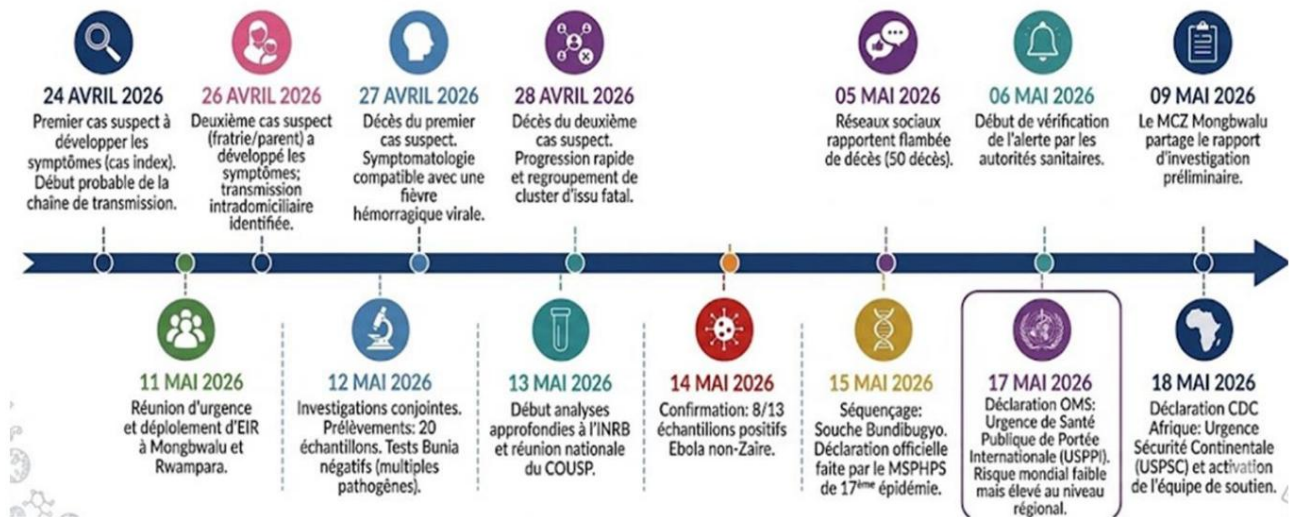


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 19, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of June 19, 2026.

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	874	201	23.0%	22 out of 36 (61.1%)	21
North Kivu	79	45	57.0%	11 out of 34 (32.3%)	2
South Kivu		1	33.3%	1 out of 34 (2.9%)	0
Total	3,956	247	25.8%	34 out of 104 (32.7%)	23

As of June 19, 2026, no new health zones were affected. The number of affected health zones remains at 34/104 (section 3.1). Furthermore, the province of South Kivu has not reported any confirmed cases since May 26, 2026 (Table 1). There has been no recent transmission in South Kivu. However, surveillance remains necessary.

3.2 Distribution by health zone (Cumulative as of 19/06/2026)

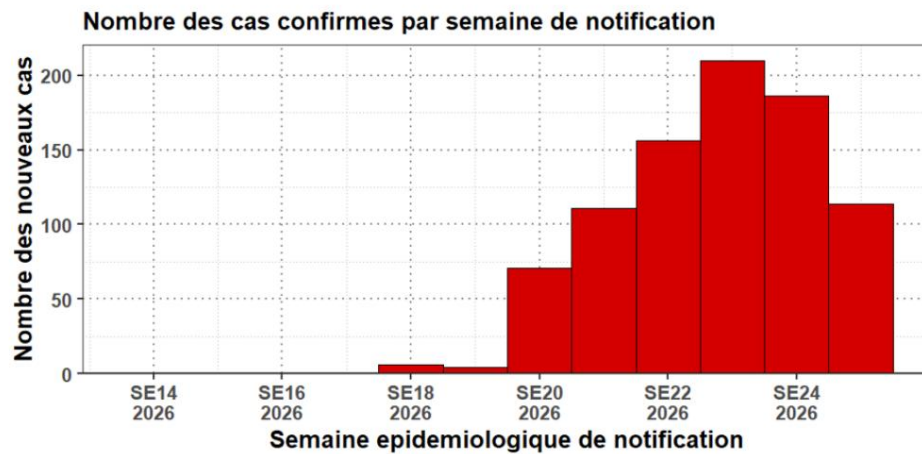
- **Ituri Province (874 cases):** Bunia (259), Rwampara (201), Mongbwalu (209), Nyankunde (79), Nizi (26), Lita (20), Komanda (10), Bambu (9), Kilo (7), Mangala (7), Tchomia (5), Damas (4), Aungba (6), Rimba (3), Aru (3), Mambasa (2), Logo (2), Drodoro (1), Gety (1), Kambala (1), Nia-Nia (1) and Fataki (1).

Seventeen (17) additional cases are awaiting precise breakdown by health zone in the province.

- **North Kivu Province (79 cases):** Butembo (25), Katwa (26), Beni (16), Oicha (3), Kalunguta (2), Kyondo (2), Goma (1), Masereka (1), Vuhovi (1), Mabalako (1) and Musienene (1).

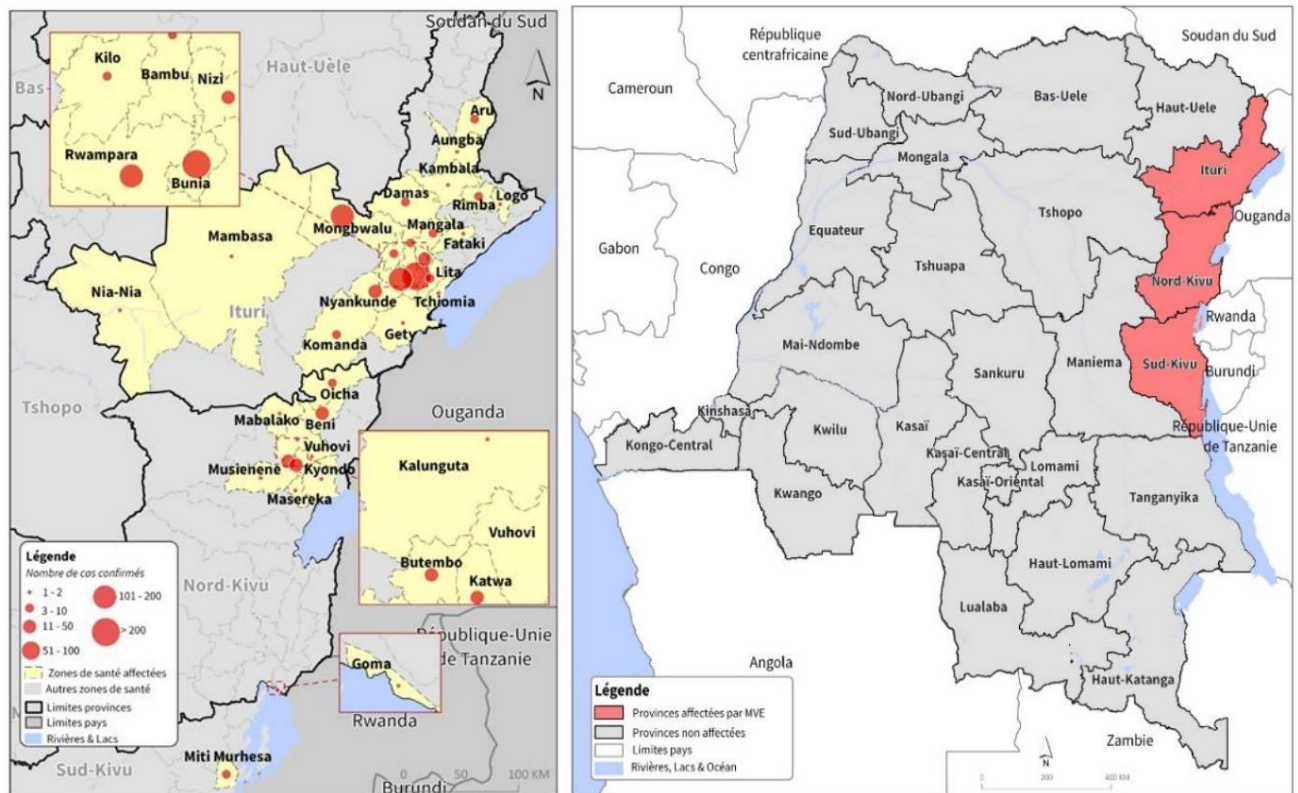
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Temporal Analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting continuous transmission of the disease within the community. Rapid geographical expansion is also observed. The epidemic remains possible if public health actions are not implemented quickly.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 858.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 19, 2026

Table 2. Confirmed cases and deaths by province and health zone (June 19, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR, %)
ITURI	874	201	23.0%
Bunia	259	42	16.2%
Rwampara	201	35	17.4%
Mongbwalu	209	89	42.6%
Nyankunde	79	7	8.9%
Nizi	26	5	19.2%
Lita	20	6	30.0%
Komanda	10	3	30.0%
Bamboo		2	22.2%
Kilo			14.3%
Mangala	9 7 7	1 4	57.1%
Tchomia	5	0	0.0%
Damascus	4	0	0.0%
Aungba	6	2	33.3%
Rimba	3	0	0.0%
Aru	3		33.3%
Mambasa	2		50.0%
Logo	2	1	0.0%
Drodro			100.0%
Gety	1 1	1 0 1 0	0.0%
Kambala	1		100.0%
Nia-Nia	1		100.0%
Fataki	1	1 1 0	0.0%
Other areas not yet identified	17	0	0.0%
NORTH KIVU	79	45	57.0%
Butembo	25	11	44.0%
Katwa	26	16	61.5%
Blessed	16	12	75.0%
Oicha	3	2	66.7%
Kalunguta	2		50.0%
Kyondo	2		50.0%
Goma		1	0.0%
Masereka		1	0.0%
Vuhovi			100.0%
Mabalako		0	0.0%
Musienene			100.0%
SOUTH KIVU	1 1 1 1 1 3	0 1 0 1 1	33.3%
Miti-Murhesa		1	33.3%
TOTAL	3,956	247	25.8%

The epidemiological situation of the 17th Ebola Virus Disease (EVD) outbreak in the Democratic Republic of Congo is progressing, with **23 new confirmed cases, including 2 deaths**, recorded in the last 24 hours. This brings the cumulative total to **956 confirmed cases** and **247 deaths**, representing an overall case fatality rate of **25.8%**.

Provincial dynamics and case concentration

The province of **Ituri** remains the absolute epicenter of the epidemic, accounting for **91.4%** of cases (n=874) and **81.4%** Deaths (n=201) represent a case fatality rate of **23.0%**. The most affected health zones are Bunia (259 cases), Rwampara (201 cases), and Mongbwalu (209 cases), encompassing slightly more than three-quarters of the reported cases (n=669) in the province. The Bunia and Rwampara health zones have moderate case fatality rates (16.2% and 17.4%, respectively), while the **Mongbwalu** health zone shows marked severity, with a case fatality rate of **42.6%** (89 deaths out of 209 cases), suggesting persistent challenges in early management or access to care. Furthermore, 17 additional cases were reported in as-yet-unidentified health zones, without associated deaths, which could indicate a geographic expansion requiring further investigation.

Severity of the epidemic in North Kivu

Cumulatively, with **79 confirmed cases** and **45 deaths**, the provincial case fatality rate reaches **57.0%**. **North Kivu** province presents the most worrying profile in terms of severity, higher than the national average.

This high case fatality rate is concentrated in the health zones of **Beni** (75.0%), **Katwa** (61.5%), and **Oicha** (66.7%). The Butembo health zone, despite having a large number of cases (n=25), has a slightly lower case fatality rate (44.0%). The presence of cases with a 100% case fatality rate in areas with low population density (Vuhovi, Musienene) is an indicator of the virus's danger in this region and the need for an immediate clinical response.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Holding of the coordination meeting focused on monitoring activities carried out in the field, chaired by the Minister of Health, Hygiene and Social Welfare in the presence of the provincial governor.
- Holding a technical meeting with technical and financial partners.
- Organization of the discharge ceremony for a patient who had recovered from Ebola virus disease at the Ebola Treatment Center Rwampara.
- Provision of 6 ambulances to strengthen the referral of patients to the various CTEs.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of June 19, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Alerts raised		233	500	ND	733
Alerts investigated		233	500	ND	733
Investigation rate		100.0%	100.0%	ND	100.0%
Alerts confirmed	Alive	75	40	ND	115
	Deceased	21	26	ND	47
Total suspected cases for today		96	66	ND	162

As of June 19, 2026, **733 alerts** were reported only in the provinces of Ituri and North Kivu, **733 (100.0%)** were investigated, **162 suspected cases (4 deaths)** were identified.

Table 4. Contact tracing of confirmed cases as of June 19, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	4,688	3,649	77.8%
North Kivu	1,698	778	45.8%
South Kivu	ND	ND	N / A
Total	6,386	4,427	69.3%

ND: Not available NA: Not Applicable

- Continued in-depth investigations and exhaustive listing and monitoring of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and monitored per case).
- 333 new contacts were listed in 9 health zones in Ituri province. These contacts are distributed in the health zones of Rwampara (155), Bunia (54), Nyankunde (39), Mongbwalu (29), Lita (20), Aungba (15), Tchomia (13), Nizi (4), Mambasa (4).

Update on checkpoint and point of entry (PoC/PoE) activities

- Three (3) alerts were notified, including 1 live alert of an escaped child from the Mongbwalu General Referral Hospital (HGR) CTE, intercepted at the **PLUTO PoC** (the family refused to return her to the CTE), which was validated, and 2 live alerts at **PoC Foner Mambasa** have been invalidated.

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC) in Ituri province, as of June 19, 2026

Indicators	Results
Proportion of activated PoCs	80.0% (44/55)
Number of travelers who passed	69,261
through, % of travelers	98.0%
screened, % of travelers who washed their	97.0%
hands, % of travelers made aware of the risks	97.0%
Number of alerts notified	3
Number of validated alerts	1
% of results available in 24 hours	0%
Number of alerts transferred to the CT/CTE or for EDS	0
Number of contacts intercepted today	0

4.3. Laboratory

- Ituri:** 101 samples collected and analyzed (positivity 20.8%; n=21).
- North Kivu :** 16 new samples collected, of which 15 were analyzed (positivity 13.3%; n=2).
- South Kivu :** No update has been shared.

4.4. Infection Prevention and Control (IPC)

Ituri: Briefing of 114 service providers was organised in the Bambu and Bunia, Komanda, Mambasa and Rwampara health zones on standard precautions (correct hand washing, wearing and removal of PPE, as well as biocleaning); 9 rings and 9 decontaminations carried out around confirmed cases in the Bunia and Rwampara health zones; EDS carried out in the Rwampara (4/5), Bunia (13/13), Mongbwalu (8/8) and Komanda (3/3) health zones.

North Kivu : Assessment of 16 exposed healthcare providers: 8 in the Katwa Health Zone (8 classified as high risk) and 8 in the Butembo Health Zone (6 classified as high risk and 2 as low risk); decontamination of 2 health facilities (triage at Makerere Health Center) after the transfer of a confirmed case and a suspected case; briefing of 104 healthcare providers on case definition and standard infection prevention and control (IPC) precautions (triage and isolation, hand hygiene, cleaning and disinfection, waste management) in the health zones of Butembo, Katwa, Beni, Karisimbi and Nyiragongo; support for the briefing of 18 morgue healthcare providers on IPC/WASH organized by the Red Cross in the Katwa Health Zone; provision of IPC/WASH supplies to 27/42 health facilities in the Butembo Health Zone with WHO support; IPC monitoring and support of 22 priority health facilities; PCI Scorecard evaluation of 11 ESS, including 10 in Beni and 1 in Butembo (average PCI score is 45.4%); In the last 24 hours, 4 EDS were carried out and 21 post-mortem samples were taken before burial.

4.5 Holistic care

- **Twelve (12) patients** with Ebola virus disease have been declared cured after obtaining negative control tests.
- Continued nutritional monitoring of patients in the CTEs: 435 hot meals were served, including 88 of the confirmed cases, 109 suspected cases, 192 PPL, 46 accompanying persons.

Table 6. Patient movement within healthcare facilities (CTE/CT/CI) as of June 19, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	286	56	ND	342
Total admissions (24h)	59	6	ND	65
Exits (24h)				
Deceased (Suspects/Confessions)	5	0	ND	5
No case	16	8	ND	24
Cured	12	0	ND	12
Escapees (Suspects/Confessions)	5	0	ND	5
Transferred to the HGR	0	0	ND	0
Total outflows	38	8	ND	46
Patients in isolation (End of Day),	307	54	ND	361
including confirmed cases	124	24	ND	148
(NC+AC) and suspected cases	183	30	ND	213
Overall occupancy rate	90.8%	76.1%	N / A	88.3%

ND: Not available NA: Not applicable

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of June 19, 2026

KEY INDICATORS	ITURI	SOUTH KIVU	NORTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	8 (40%)	1	0
Confirmed cases currently being monitored that have received interventions SMSPS	47 (37.6%)	5 (100%)	0
New suspected cases that benefited from SMSPS interventions	2 (2.7%)	3 (100%)	2 (100%)
Suspected cases under follow-up that have received interventions SMSPS	36 (20.7%)	11 (100%)	8 (100%)
Laboratory results have been announced.	25	5	0
Including positive ones	10	1	0
Number of children separated in daycare who benefited from interventions SMSPS	3	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	0	16
Number of accompanying persons at the CTE who benefited from interventions SMSPS	42	12	0
Number of PPLs who benefited from SMSPS interventions	16	5	21
Number of household and community members who benefited from SMSPS interventions	35	32	0
Number of EDS supported	1	0	0

4.6 Continuity of care

- 20 midwives and nurses from the NYAKUNDE health zone were trained in sexual and reproductive health in the context of Ebola.

4.7 Risk communication and community engagement

Ituri: Community dialogue with 22 people (19 men and 3 women) on adherence to Ebola prevention measures, care, community alert reporting, and EDS (Emergency Health Dependency) in the Bunia health zone; 5,801 people affected (3,075 men and 2,726 women); dissemination of public service announcements and messages in 18 media outlets on EVD preventive measures; hosting 4 radio programs on EVD preventive measures and community-based surveillance in 4 different media outlets.

Support for other pillars : PCI : implementation of 8 EDS at CTE/CME and CTE Elikya.

North Kivu: 11,735 people sensitized, 7,732 households visited and the deployment of 1,631 community relays; the involvement of 564 community leaders and the production of 18 radio broadcasts and community dialogues contribute to strengthening the dissemination of prevention messages and improving the acceptance of response interventions.

4.8. Logistics

- Receipt of PCI kits.
- Delivery of PCI and MEG inputs to the health zones of Bambu, Kilo, Mongbwalu and to the HGR Bunia treatment center CTE CME Bunia and CSR SOTA/ZS Nyankunde.

- Delivery of PPE kits to the Rwampara health zone.
- Delivery of PCI inputs to CH Mudzi Maria/ZS Bunia as well as to the PCI Wash pillar.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : Awareness-raising of 1,254 (567 women and 687 men) on PEAHS with the faithful of the Anglican church in the Bunia health zone; 43 participants (22 women and 21 men) members of the local security committee of the commune of Shari.

South Kivu : Awareness-raising of 31 participants (6 women and 25 men) from the community of Kahungu, Buhandahanda and Kavumu in the Miti-Murhesa health zone.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity in CTE and saturation in Ituri (88.3%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3 Poor performance in contact tracing (69.3% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4. Low reporting of alerts in certain affected zones	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not localized or unclassified (~17 cases)	Major gaps in the reconstruction of transmission chains	Conducting a data reconciliation, Remove any potential duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.). Active verification in the field.
6 Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (78 cases) HCW (including 18 deaths, case fatality rate 23.1%)	Urgent supply, strengthening of IPC training
8 Stockpile of unanalyzed samples (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (57.0%)	Deploy laboratory catch-up plan, strengthen diagnostic capabilities
9 Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10. Insufficient coordination and low utilization of COUSP frameworks	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, operationalize the COUSP

11. Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12 Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13. Continuity of Essential Services (CEHS) Compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14. High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

Installation of a mobile laboratory in the Aru health zone, Ituri province.



**FOR ALL
INFORMATION
ADDITIONAL,
PLEASE CONTACT**

For the National Institute of Public Health (INSP) of the DRC

The Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: diedonne.mwamba@insp.cd

Tel.: +243 816 040 145

The COU-SP Coordinator

Professor NGANDU Christian

Email: Christian.ngandu@insp.cd

Tel.: +243998091915

The Incident Manager

Professor Pierre AKILIMALI

Email: pierre.akilimali@insp.cd

Tel: +243815800288

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